

National Coverage Determination (NCD)

Ambulatory Blood Pressure Monitoring

20.19

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Tracking Information

Publication Number

100-3

Manual Section Number

20.19

Manual Section Title

Ambulatory Blood Pressure Monitoring

Version Number

2

Effective Date of this Version

07/01/2003

Ending Effective Date of this Version

07/02/2019

Implementation Date

07/01/2003

Description Information

Benefit Category

Diagnostic Tests (other)

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

Ambulatory blood pressure monitoring (ABPM) involves the use of a non-invasive device which is used to measure blood pressure in 24-hour cycles. These 24-hour measurements are stored in the device and are later interpreted by the physician.

Indications and Limitations of Coverage

ABPM must be performed for at least 24 hours to meet coverage criteria.

ABPM is only covered for those patients with suspected white coat hypertension. Suspected white coat hypertension is defined as

1. Office blood pressure >140/90 mm Hg on at least three separate clinic/office visits with two separate measurements made at each visit;
2. At least two documented blood pressure measurements taken outside the office which are <140/90 mm Hg; and
3. No evidence of end-organ damage.

The information obtained by ABPM is necessary in order to determine the appropriate management of the patient. ABPM is not covered for any other uses. In the rare circumstance that ABPM needs to be performed more than once in a patient, the qualifying criteria described above must be met for each subsequent ABPM test.

For those patients that undergo ABPM and have an ambulatory blood pressure of <135/85 with no evidence of end-organ damage, it is likely that their cardiovascular risk is similar to that of normotensives. They should be followed over time. Patients for which ABPM demonstrates a blood pressure of >135/85 may be at increased cardiovascular risk, and a physician may wish to consider antihypertensive therapy.

Claims Processing Instructions

[TN 109 \(Medicare Claims Processing\)](#) 

[TN 10073 \(Medicare Claims Processing\)](#) 

Transmittal Information

Transmittal Number

168

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/Downloads/R168CIM.pdf> 

Revision History

11/2016 - This change request (CR) is the 9th maintenance update of ICD-10 conversions and other coding updates specific to national coverage determinations (NCDs). The majority of the NCDs included are a result of feedback received from previous ICD-10 NCD CR7818, CR8109, CR8197, CR8691, CR9087, CR9252, CR9540, and CR9631. Some are the result of revisions required to other NCD-related CRs released separately.

Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent, quarterly releases as needed. No policy-related changes are included with these updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. ([TN 1753](#)) (CR9751)

08/2016 - This change request (CR) is the 9th maintenance update of ICD-10 conversions and other coding updates specific to national coverage determinations (NCDs). The majority of the NCDs included are a result of feedback received from previous ICD-10 NCD CR7818, CR8109, CR8197, CR8691, CR9087, CR9252, CR9540, and CR9631. Some are the result of revisions required to other NCD-related CRs released separately.

Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent, quarterly releases as needed. No policy-related changes are included with these updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. ([TN 1708](#)) (CR9751)

09/2012 - CMS translated the information for this policy from ICD-9-CM/PCS to ICD-10-CM/PCS according to HIPAA standard medical data code set requirements and updated any necessary and related coding infrastructure. These updates do not expand, restrict, or alter existing coverage policy. Implementation date: 01/07/2013 Effective date: 10/1/2015. ([TN 1122](#)) ([TN 1122](#)) (CR 7818)

03/2003 - Specified physician is required to perform interpretation of data obtained through ABPM, but no requirements regarding setting where interpretation performed. Effective and implementation dates 07/01/2003. ([TN 168](#)) (CR 2625)

12/2001 - Changed status from non-covered to covered and clarified conditions under which ABPM is covered. Effective and implementation dates 04/01/2002. ([TN 149](#)) (CR 1985)

National Coverage Analyses (NCAs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with NCAs, from the National Coverage Analyses database.

- [Original Consideration for Ambulatory Blood Pressure Monitoring \(CAG-00067N\)](#)
- [First reconsideration for Ambulatory Blood Pressure Monitoring \(CAG-00067R\)](#)
- [Second reconsideration for Ambulatory Blood Pressure Monitoring \(ABPM\) \(CAG-00067R2\)](#)

Additional Information

Other Versions

Title	Version	Effective Between	
Ambulatory Blood Pressure Monitoring	3	07/02/2019 - N/A	View
Ambulatory Blood Pressure Monitoring	2	07/01/2003 - 07/02/2019	You are here
Ambulatory Blood Pressure Monitoring	1	04/01/2002 - 07/01/2003	View